MEDSTAR: THE AUTOPSY OF A CIVIC MURDER

By J. Jonah Jameson Editor-in-Chief, The Daily Bugle (Fort Worth Bureau)

1. Lead: The Death of the System

They didn't kill MedStar Mobile Healthcare with a gun. They didn't do it with a knife. They did it with a spreadsheet, a consultant's report, and a vote in a council chamber that smelled of floor wax and cowardice.

On May 21, 2024, the Fort Worth City Council raised eleven hands and lowered thirty-eight years of operational excellence into the ground. Eleven to zero. A unanimous execution. They called it a "transition." They called it a "dissolution." They used the sanitized, sterile language of bureaucracy to mask the stench of a body decomposing in real-time. But make no mistake: this was a murder.

For nearly four decades, MedStar was the pulse of this city. It was the blue star on the white ambulance that showed up when your heart stopped, when your lungs filled with fluid, when the drunk driver crossed the center line on I-35. They were the grunts. The street-level operators. They weren't the ones marching in the parades with the shiny red trucks and the Dalmatians. They were the ones kneeling in the gutter fluids and the broken glass, stabilizing cervical spines while the rest of the city slept. They were the "UCALL, WE HAUL" brigade, the nameless faces who carried the weight of the city's trauma on their backs.

And for their trouble, they were liquidated.

The official story—the one printed on the glossy handouts at City Hall and parroted by the spokespeople—is that MedStar was "financially unsustainable." They tell you the model was broken. They tell you that response times were slipping, that costs were rising, and that the only way to save the soul of the city's emergency services was to hand it over to the Fort Worth Fire Department. They wave a consultant's report in your face like a preacher waving a bible, shouting that this $85 million takeover is the path to salvation.

I've been in the news game too long to buy the first draft of history. I've seen too many good organizations gutted to feed a political machine. I've read the reports. I've looked at the numbers. I've traced the money from the "subsidy" denials to the "deficit" approvals.

This isn't about patient care. If it were about patient care, you don't take the only agency in the country to win the "EMS10" Innovator award *twice* and dismantle it for spare parts. You don't take a system that saved the healthcare infrastructure over $30 million through community paramedicine and throw it in the scrap heap.

No, this is about power. This is about budgets. This is about a union that wanted a bigger piece of the pie and a city council too terrified to tell them no.

The city is paying a projected $85 million a year now for what MedStar used to do for free. Let that sink in. We traded a self-sustaining public utility for a tax-funded monolith, and we did it while 589 people held pink slips in their shaking hands.

This is the obituary of MedStar. But it's also an autopsy of the Fort Worth political machine. We're going to cut this thing open. We're going to look at the organs. And we're going to find out

what really killed the best EMS system in Texas.

2. The Origin Sin

To understand the murder, you have to understand the birth. MedStar didn't just appear out of the ether. It was forged in the fires of 1986, created by the City of Fort Worth to solve a problem that municipalities have been trying to solve since the invention of the siren: how to pay for an ambulance without raising taxes.

They called it the "Public Utility Model" (PUM). It sounds boring. It sounds like something you read about in a zoning manual or a water district charter. But in 1986, it was radical. It was the "Fort Worth Way" before that was a marketing slogan.

The idea was simple: EMS should be a business, but a business owned by the public. It would survive on what it billed. If you called an ambulance, your insurance paid for it. If you didn't, you didn't pay a dime. No tax levy. No line item on your property tax bill. The people who used the system paid for the system.

It was the ultimate free-market compromise. The city got a world-class ambulance service for zero dollars in subsidy. For decades, MedStar operated on a "eat what you kill" basis. They billed Medicare. They billed Blue Cross. They hustled. They ran a lean, mean operation because they *had* to. They didn't have the luxury of a guaranteed budget from the General Fund. If they wanted new tires for the rigs, they had to earn the money to buy them.

This was the Origin Sin.

By making MedStar self-sufficient, the city set a trap for the future. They created a system that was financially isolated. The Fire Department? They get paid whether there's a fire or not. The Police Department? They get their budget regardless of the crime rate. But MedStar? MedStar only ate if the billing codes cleared.

For thirty years, it worked. It worked beautifully. It worked so well that the city forgot that emergency medicine costs money. They got used to the free lunch. They started to believe that EMS was a magical service that just *happened*, like the sun coming up or the Trinity River flowing.

But the economy changes. Healthcare changes. And when the math stopped working, the city didn't look at MedStar as a partner to be supported. They looked at it as a tenant who was late on the rent.

The Public Utility Model relies on a delicate balance. You need a mix of patients. You need the private insurance payouts to offset the Medicare payouts and the uninsured patients. You need volume. And you need a reimbursement system that actually pays for the cost of care.

But then came the squeeze. Medicare rates flattened. Insurance companies got stingier. The cost of diesel went up. The cost of epinephrine went up. The cost of a human being willing to work a 12-hour shift pulling bodies out of wrecked cars went up.

MedStar saw it coming. They didn't hide it. Since 2019, they were sounding the alarm. They told the board: the runway is running out. We can't keep doing this on billing alone. We need a safety net.

But the city wasn't listening. They were too busy enjoying the free ride.

The Origin Sin of MedStar was competence. They were so good at managing money, so good at squeezing efficiency out of a stone, that they convinced the politicians they didn't need help. They made it look easy. And in politics, if you make it look easy, nobody thinks you're doing any work.

So when the bill finally came due—when the gap between revenue and reality opened up—the

city didn’t see a structural problem with healthcare economics. They saw a "failure" of the MedStar model.
They didn’t want to fix the funding. They wanted to erase the debt. And the easiest way to erase the debt was to erase the organization.

3. The Era of Competence

Before we shovel the dirt, let’s look at the record. Let’s look at what they destroyed. You don’t hear this from the City Manager’s office. You don’t hear this from the Fire Union reps giving quotes to the *Star-Telegram*. But MedStar wasn’t just "good." It was elite.
This wasn't a "you call, we haul" taxi service. This was a high-performance clinical machine that the rest of the world envied.
We’re talking about an agency that was named the "EMS System of the Year" by the Texas Department of State Health Services. Not once. Multiple times. We’re talking about an agency that won the "Paid EMS System of the Year" from NAEMT. We’re talking about the only agency in the United States to be named an "EMS10 Innovator" by JEMS Magazine *twice*.
Why? Because they weren’t just driving taxis with sirens. They were doing actual medicine.

The Innovation Engine: Mobile Integrated Healthcare (MIH)

MedStar didn't just respond to emergencies; they prevented them. They invented the "Mobile Integrated Healthcare" program. This is the stuff that saves lives *and* tax dollars. They realized that not every 911 call needs an ambulance ride to the ER. Grandma doesn’t need a $2,000 transport because she forgot her medication; she needs a paramedic to come over, check her vitals, organize her pillbox, and make her a cup of tea.
MedStar did that. They created a "StarSaver" program. They created high-utilizer groups to identify the people calling 911 fifty times a year and get them actual social services instead of another pointless trip to the hospital waiting room.
The numbers are in the files, if you care to look. That program prevented 10,584 ambulance transports. It stopped 7,740 ER visits. It saved the healthcare system $30.2 million since 2009.
$30.2 MILLION.
That’s money that stayed in the pockets of the public. That’s ER beds that stayed open for heart attacks and gunshot wounds because MedStar kept the non-emergencies at home.
The Agency for Healthcare Research and Quality—the feds—published MedStar’s programs as a model for the nation five years in a row. People flew in from 46 states and 6 foreign nations to visit Fort Worth and see how MedStar did it. They came to our city to learn how to save lives. Fort Worth was the Mecca of high-performance EMS. We were the gold standard.

The Culture of the Blue Star

And the staff? These weren’t transients. These were career medics. People like Richard Ponikiewski, who was there on Day One in 1986 and was still there decades later. People who knew every shortcut in Haltom City, every nursing home in White Settlement, every dangerous intersection in Saginaw.
They had a culture. They had an identity. They weren't firefighters who got stuck on the ambulance rotation because they drew the short straw. They were medics. That’s all they wanted to be. That’s all they cared about.

They drilled on EKGs while the fire guys drilled on hydrants. They studied pharmacology while the fire guys studied ventilation tactics. That's not an insult to the fire department; it's a fact of specialization. You want a specialist when you're dying. You don't want a jack-of-all-trades. You want the guy who has done ten thousand intubations, not the guy who did five in training and spent the rest of the year waxing the ladder truck.

MedStar transported an average of 296 patients a day. They drove three million miles a year. They used 443,000 gallons of fuel. They were a machine. A grinding, sweating, life-saving machine.

And they did it with "economic efficiency" that would make a Wall Street accountant weep. They ran a fleet of 65 ambulances across 436 square miles for a fraction of what other cities paid.

This was the Era of Competence. It was the era where Fort Worth punched above its weight. We had a world-class system that cost the taxpayer *nothing* in direct subsidies for most of its history.

But competence is a double-edged sword. When you do your job too well, people stop appreciating the difficulty. They assume it's magic. And when the magic stops—when the economics shift and the bill comes due—they don't say "thank you for the thirty years of free service."

They say, "Why can't you do it for free anymore?"

And then they reach for the shovel.

4. The Slow Bleed

You want to know how a system dies? It doesn't happen overnight. It happens by a thousand cuts. It happens in the spreadsheets.

The narrative they sold you is that MedStar "failed." That's a lie. MedStar didn't fail; the economy of healthcare collapsed around it, and the City of Fort Worth refused to reinforce the foundation.

Here is the math they don't put on the billboards:

MedStar's revenue was $60 million a year. Sixty percent of that came from Medicare and Medicaid. You know what Medicare pays for an ambulance ride? Peanuts. It's below the cost of the diesel, let alone the medic's salary.

For years, MedStar made up the difference with private insurance payments. But insurance companies got smarter. They started denying claims. They started negotiating lower rates. The "cost shift"—the game of charging Blue Cross to pay for Medicare patients—stopped working.

Then came COVID-19.

The pandemic broke everything. It broke the supply chains. It broke the staffing pipelines. It broke the morale of the workforce. But mostly, it broke the budget.

Wages skyrocketed. You couldn't hire a paramedic for $15 an hour anymore, and you shouldn't have been able to in the first place. MedStar had to raise wages to keep people. Fuel costs doubled. The price of ambulances went up 30%.

The Request Denied

MedStar went to the city. They went to the "member cities"—the 13 other towns that rode on Fort Worth's coattails. They said, "We need help. We need a subsidy."

They asked for **$6 million**.

Six million dollars. In a city budget of over $2.6 billion, that is a rounding error. That is less than

the city spends on "consultants" for park redesigns.
But the city balked. The City Council looked at that $6 million request like it was an insult. They were so addicted to the "free" model that the idea of paying for EMS felt like a failure.
They didn't see the $30 million saved by the MIH programs. They didn't see the decades of zero-cost service. They just saw a red line on a spreadsheet.

The Vultures Circle

Meanwhile, the vultures started circling.
The Fort Worth Firefighters Association—Local 440—saw an opening. They've been eyeing the ambulance work for years. Why? Because fires are down. Building codes work. Sprinkler systems work. The number of actual structural fires in a modern city is a fraction of what it was in 1980.
But medical calls? Medical calls are up. If you want to justify a massive fire department budget, if you want to keep the station counts high and the staffing levels robust, you need call volume. And the call volume is in the ambulances.
So you have a perfect storm. MedStar is bleeding cash because of global economic trends. The city is stingy with the subsidy. And the Fire Union is whispering in the ears of the Council members: "We can do it better. We can do it cheaper. Just give it to us."

The "Deficit" Hypocrisy

It was a lie, of course. They couldn't do it cheaper. The projected budget for the new Fire-based EMS is $85 million. The revenue is only $65 million. That's a **$20 million deficit** right out of the gate.
Wait a minute.
MedStar asked for $6 million to stay alive. The city said no. The Fire Department asks for a system that will run a $20 million deficit, and the city says "Yes, please."
Do the math. This wasn't about money. If it was about money, they would have cut MedStar a check for $6 million and called it a day.
This was about the bleed. They let MedStar bleed out. They watched the response times tick up—because MedStar couldn't afford to staff enough trucks—and instead of throwing them a lifeline, they pointed at the clock and said, "Look! They're failing!"
They starved the horse and then shot it for limping.
The "Slow Bleed" wasn't an accident. It was a strategy. By refusing to subsidize the Public Utility Model during a global inflation crisis, the city ensured MedStar would look incompetent. They manufactured a crisis so they could sell the solution.
And the solution just happened to be the one the Fire Union wanted all along.

5. The Killing Blow

Every murder needs a weapon. In this case, the weapon was a report, and the accomplice was a union.
The city hired **Fitch & Associates**. They paid them $182,500 to "study" the system.
Let me tell you about Fitch & Associates. In the EMS world, hiring Fitch to decide if you should switch to a Fire-based system is like hiring Colonel Sanders to decide if you should eat chicken. They have a reputation. They write the reports that justify the takeovers. They use "industry

standards" that align perfectly with the expansionist goals of fire departments.
The report dropped in April 2024. And it was the nail in the coffin.
It painted a picture of a system in collapse. It highlighted the response time failures—failures caused by the funding starvation the city itself orchestrated. It offered "options," but the deck was stacked.
Option A: Keep MedStar (but it will cost money). Option B: Third Service (too expensive). Option C: Fire-Based EMS (The Golden Child).
They sold Option C as the "sustainable" model. They claimed it would improve response times. They claimed it would be "more efficient" because of shared infrastructure.
The City Council swallowed it whole. On May 21, 2024, they voted 11-0 to dissolve MedStar. Eleven to zero. Not a single dissenting voice. Not one council member stood up and asked, "Wait, why are we paying $20 million for a deficit when we could fix MedStar for $6 million?"

The Martyrdom of Chief Davis

But the real killing blow didn't happen in the council chamber. It happened in the backrooms with **Local 440**.
The Firefighters Association flexed its muscle like never before. This was their Super Bowl. They wanted those 600 EMS jobs under their umbrella. They wanted the Civil Service protection. They wanted the dues.
But there was a problem. **Fire Chief Jim Davis.**
Davis was a reformer. He was the guy tasked with making this transition work. But he made a fatal mistake: he actually expected firefighters to be part of the medical solution.
In late August 2024, Chief Davis announced that *all* firefighters would be required to undergo driver training to serve on ambulances. He wanted a unified department. If you're Fire-EMS, you do EMS. He knew that to handle the volume of 500-600 calls a day , he needed every hand on deck. He couldn't just rely on the incoming MedStar refugees.
The union revolted.
They didn't want to ride the box. They wanted the *control* of the box, but they didn't want the *work* of the box. The union threatened a "Vote of No Confidence" in Chief Davis.
They cited "safety violations." They cited "rushed implementation." They threw everything at the wall. But the core grievance was clear: *Don't make us drive the ambulance.* They claimed it "greatly increases workload". Imagine that. Working on an ambulance increases your workload.
The city caved. The city always caves.
They placed Chief Davis on "administrative leave" in September—a coward's way of firing a man without firing him. Then, in October, Davis "resigned".
They paid him **$99,999** to go away quietly and drop his whistleblower claims.
Think about that. The man tasked with leading this massive transition—the biggest change in public safety in forty years—was decapitated by the union weeks before the finalization because he tried to make the system efficient.
That was the Killing Blow. It wasn't just the vote to dissolve MedStar. It was the political assassination of the one man who might have held the Fire Department accountable for the actual work.
With Davis gone, the Union ran the table. They got the MOU they wanted. They got the Civil Service structure they wanted. They got the jobs, the budget, and the power.
And MedStar? MedStar was just the carcass they stepped over to get to the podium.

6. The Human Cost

Now we get to the part that keeps me up at night. The people. Systems don't bleed. People do. When the gavel came down, 589 employees at MedStar received WARN notices.
These are men and women who gave their youth to this city. They ruined their backs lifting 300-pound patients down narrow stairwells. They missed Christmases. They missed their kids' soccer games. They developed PTSD from seeing things that would make a council member vomit in a wastebasket.
And what did they get?
"Reapply for your job."
That was the deal. The city "invited" them to apply to the Fort Worth Fire Department. But not as equals. Oh no.

The Caste System: Single vs. Dual Role

They created a caste system. "Single Role" vs "Dual Role".
The firefighters are "Dual Role." They get the glory. They get the traditional respect. The MedStar refugees? They are "Single Role." They are the ambulance drivers. They are the second-class citizens in the firehouse.
The union talks a good game about "brotherhood," about "workplace equality" and "mixers and dinners" to blend the cultures. But you can't erase a caste system with a pizza party. The new MOU specifies that Single Role employees are there for EMS. They are distinct.

The Pension Robbery

And the pensions? This is the invisible knife.
MedStar employees had a pension system. It wasn't perfect, but it was theirs. It was a defined benefit plan, later frozen and shifted, but it had structure. When they moved to the city, the structure changed.
The snippets tell us the city "hopes" to bridge the gap, but let's be real. You don't seamlessly transfer a private-public authority pension into a municipal civil service fire pension without losers. The city faced a "$20 million gap" largely due to staffing costs, pensions, and healthcare benefits.
Who pays for that gap? The taxpayers, yes. But also the employees who find their years of service don't quite calculate the same way in the new formula. A medic with 20 years at MedStar—a guy like **George Church** who served 35 years —is suddenly navigating a new bureaucracy where his tenure is a line item to be negotiated, not a badge of honor.

The Ones Left Behind

And let's talk about the ones who didn't make the cut. The 589 WARN notices didn't perfectly match the 600+ hires. There are people who fell through the cracks.
The billing department. The administrative staff. The mechanics who knew how to fix a Type III ambulance blindfolded.
The City of Fort Worth has its own billing department. They have their own mechanics. They didn't need MedStar's back office. So those jobs are just *gone*. Vaporized.
These are 60, 70 people who weren't medics but were the backbone of the operation. They

didn't get a "swearing in" ceremony. They got a cardboard box and a handshake.
These are mortgages that won't get paid. These are college funds that just evaporated.
The city calls this "efficiency." I call it throwing families into the wood chipper to balance a spreadsheet.

7. The Official Story vs. The Real Story

Let's put the lies side-by-side with the truth. I'm tired of the euphemisms. I'm tired of the press releases.

DATA EXHIBIT A: THE COST OF "SAVING" MONEY

Metric	MedStar (The "Failed" Model)	Fort Worth Fire EMS (The "Solution")
Annual Budget	~$60 Million (Self-Funded)	$85 Million (Projected)
City Subsidy Request	$6 Million (Denied)	$20 Million+ Deficit (Approved)
Revenue Source	Billing/Insurance (User Pays)	Taxpayer Subsidized
Employees	~600 Dedicated Medics	Mixed (Single/Dual Role Caste System)
Clinical Awards	EMS10 (2x), NAEMT System of Year	*None yet*
Leadership Fate	Dissolved	Chief Forced to Resign by Union

The Lie of Sustainability

The Lie: "MedStar was financially unsustainable." **The Truth:** MedStar needed a $6 million subsidy to survive the post-COVID inflation that hit every industry in the world. The city refused it. Then the city approved a Fire-based model with a $20 million *annual* deficit. *Insight:* The "sustainability" argument is a fraud. The city is willing to lose *more* money on the Fire Department than they ever would have spent saving MedStar. Why? Because paying the Fire Department is politically popular. Paying a "contractor" is not.

The Lie of Response Times

The Lie: "This will improve response times." **The Truth:** Maybe. But at what cost? The Fitch report claims the Fire model will be faster because of station locations. But the Fire Union just ousted the Chief for trying to make firefighters drive the ambulances. If the firefighters aren't fully engaged in the EMS mission, if they view the ambulance as a "penalty box," those response times aren't going to improve. You can't optimize a workforce that resents the work.
The Lie: "It's a merger." **The Truth:** It's a hostile takeover. MedStar didn't want this. The employees didn't vote for this. The "member cities" (Haltom City, White Settlement, etc.) had no real power. Fort Worth held 5 seats on the board; the other cities shared one. This was Fort Worth bullying its neighbors into a system that benefits the Fort Worth Fire Union.

The Lie of Standard of Care

The Lie: "Standard of Care will be maintained." **The Truth:** MedStar was a specialist agency.

They won awards for clinical excellence. Fire departments, by definition, are generalists. The "Clinical Focus" inevitably dilutes when you are also worrying about hazardous materials, structural collapse, and cat rescues. You are trading a brain surgeon for a very talented GP. It might be fine for a scraped knee. It might not be fine for a complex cardiac event.
The Lie: "Chief Davis resigned for personal reasons." **The Truth:** Chief Davis was forced out because he stood up to Local 440. He tried to integrate the departments for real, and the union cut his legs off. His resignation is the smoking gun that proves who is really running this city. It's not the City Manager. It's not the Mayor. It's the Union Hall.

8. Closing Indictment

So here we are. The transition is complete. The blue ambulances are being repainted red. The uniforms are changing. The "MedStar" signs are coming down off the building on Alta Mere Drive.
The City of Fort Worth has its new toy. The Fire Union has its new members (and its new dues). The politicians have their "solution" to a problem they created.
But what have we lost?
We lost the truth. We lost the idea that public service should be about *results*, not politics. We lost an agency that proved—for thirty-eight years—that you could run a government service with the efficiency of a business and the heart of a charity.
We lost the check and balance.
When EMS is separate from Fire, they keep each other honest. When you have a dedicated medical director and a dedicated CEO, their only job is patient care. Now, it's all one big family. Who investigates the failures? Who holds the line on the budget? It's all internal now. The transparency is gone.
And we lost the moral high ground.
I look at the empty parking lot where the MedStar crews used to shift change. I see the ghosts of the men and women who worked there. I see the thousands of lives they saved—not for a pension, not for a parade, but because it was the job.
They deserved better than a consulting report and a layoff notice.
They deserved a city that fought for them, not one that sold them out to the lowest bidder with the loudest lobbyist.
Fort Worth, you have your new EMS system. You paid $85 million for it. You sacrificed a good man in Chief Davis for it. You buried a world-class agency for it.
I hope it's worth it.
But the next time you hear a siren wail in the night, don't look for the Star of Life. Look for the smoke. Because that's all that's left of the truth in this town.
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DATA EXHIBIT B: THE INNOVATION GRAVEYARD

Projects killed or endangered by the dissolution of the MedStar Public Utility Model.

- **Mobile Integrated Healthcare (MIH):**
  - *Impact:* Reduced 911 calls by frequent users.
  - *Savings:* $30.2 Million to healthcare system since 2009.
  - *Fate:* Absorbed into Fire bureaucracy; likely to be deprioritized in favor of transport volume.

- **9-1-1 Nurse Triage:**
  - *Impact:* Navigated low-acuity calls away from ambulances.
  - *Fate:* Uncertain. Fire models typically prioritize "rolling trucks" to justify staffing levels.
- **The "StarSaver" Membership:**
  - *Impact:* Protected residents from out-of-pocket costs.
  - *Fate:* Discontinued/Restructured under municipal billing rules.

To the families of the MedStar 589: This record stands for you. The city may have buried the name, but they cannot bury the work you did.

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